

Delirium & Altered Mental Status

Inpatient approach for IM residents — diagnose, find the cause, treat without sedatives

Step 1 — Diagnose with the CAM

Acute, fluctuating inattention — and find the hypoactive form

CAM feature	Required?
1. Acute onset & fluctuating course	Yes
2. Inattention	Yes
3. Disorganized thinking	Either 3
4. Altered level of consciousness	or 4

PEARL Diagnosis = 1 AND 2, plus 3 OR 4. Hypoactive delirium is the most common and most missed.

Step 2 — It's a symptom: find the cause

DELIRIUM(S) — meds and infection first

- D — Drugs (anticholinergics, benzodiazepines, opioids, steroids)
- E — Electrolytes / Endocrine | L — Lack of drugs (withdrawal) / pain
- I — Infection | R — Reduced sensory input
- I — Intracranial | U — Urinary / fecal retention
- M — Metabolic (hypoxia, hypercarbia, glucose, organ failure) | S — Sleep / Subdural
- Imaging / LP / EEG only if focal signs, trauma, or unexplained

PEARL Review the med list before ordering a head CT — it's higher-yield and free.

Step 3 — Treat: non-pharmacologic first

The HELP bundle is the evidence base

Lever	Action
Reorient	Clocks, calendars, family, glasses, hearing aids
Mobilize	Out of bed; remove tethers (lines, catheter, restraints)
Sleep	Protect nights, cluster care, daylight, fewer alarms
De-prescribe	Stop deliriogenic drugs; treat pain, constipation, retention

PEARL 2025 APA: antipsychotics neither prevent delirium nor shorten it.

Medications — the narrow roles

Antipsychotic

- Only for severe agitation threatening safety/care
- After non-drug measures fail
- A short bridge, NOT a treatment
- Avoid in Parkinson's / Lewy body

Benzodiazepine

- Only for alcohol / sedative withdrawal
- Otherwise CAUSES delirium
- Not for routine agitation
- Not scheduled for sleep

PEARL If you reach for haloperidol you're managing behavior — the cause and bundle do the real work.

Prevent & protect — and what to avoid

DO

- Screen with CAM/4AT (catch hypoactive)
- Prevent in at-risk patients with HELP levers
- Sensory aids in; mobilize; protect sleep
- De-prescribe; treat pain/retention

AVOID

- Antipsychotics to prevent/'treat' delirium
- Benzodiazepines (except withdrawal)
- Physical restraints
- Unnecessary catheters and lines

PEARL Restraints and catheters worsen and prolong delirium — remove the tethers.

Take-home points

- Delirium = acute, fluctuating inattention — diagnose with CAM, find the hypoactive form
- It's a symptom: hunt the cause (meds + infection first)
- Neuroimaging/LP/EEG only for focal signs, trauma, or unexplained cases
- Multicomponent non-pharmacologic care is the evidence base
- Antipsychotics don't prevent/shorten it; benzodiazepines only for withdrawal